

CTA CORONARY ARTERIES: 10/31/2024 11:20 AM

COMPARISON: None.

PROCEDURE COMMENTS:

Premedication: None

Intravenous Beta-blocker: 5 mg metoprolol administered

Nitroglycerine: None

Image Acquisition by Series (and Radiation Doses in CTDIvol):

1. Coronary Angiogram (mGy)

Estimated radiation doses are based on a 32 cm body phantom reference.

Estimated cumulative dose or total dose-length-product (DLP) is: Not available at time of dictation.

Intravenous Contrast Medium Administered: mL of Isovue 370

Cardiac Gating: Coronary Angiogram was performed using FLASH protocol and ECG-based tube current modulation targeting diastole with retrospective cardiac gating.

Image Reconstruction: Axial images of the Coronary Angiogram acquisition were reconstructed at thin (approximately 0.5-1 mm) slice thicknesses at the targeted R-R interval (70%). Lower resolution axial images were also reconstructed of the entire R-R interval (0-90%).

3D reformations consisting of curved and multiplanar reformations, maximum intensity projections, and volume rendered images were performed on an independent workstation and corroborate the findings made in the report of the source images.

FINDINGS:

Prior Surgery/Intervention: None.

CORONARY ARTERIES:

Left Main: Arises from its expected location off the left coronary sinus. Patent without significant narrowing. Circulation is left dominant.

Left Anterior Descending: Patent without significant narrowing.

Left Circumflex: Patent without significant narrowing.

Right: Abnormal origin from the left coronary cusp, likely from separate origin just adjacent to the left main origin. The proximal right coronary artery appears significantly narrowed with a relatively long interarterial course. There is limited opacification of the distal right coronary artery, likely flow-related in setting of proximal stenosis.

EXTRACORONARY CARDIOVASCULAR

Heart: Normal in size. Normal in architecture. There is mild right ventricular hypertrophy.

Pericardium: No pericardial effusion.

Visualized Aorta: No significant abnormality.

Pulmonary Arteries: Not enlarged. No obvious central filling defects on this non-dedicated study to suggest pulmonary embolism.

OTHER

Medical Devices: None.

Mediastinal/Hilar Lymph Nodes: No pathologically enlarged lymph nodes.

Other Mediastinal Structures: No significant abnormality.

Lung Parenchyma: Lungs are clear.

Airways: Central airways are clear.

Pleura: No pleural effusions.

Chest Wall: No significant abnormality.

Visualized Abdomen: No significant abnormality on this limited, non-dedicated study.

Bones: No aggressive osseous lesions.

IMPRESSION:

1. Anomalous origin of the right coronary artery from the left coronary cusp with an interarterial and likely intramural course given significant narrowing along the proximal RCA. Slow filling of distal RCA.
2. Left dominant coronary circulation.
3. Mild right ventricular hypertrophy.

Dictated by Resident: Evan Johns - 10/31/2024 11:41 AM

Interpreted by Attending Radiologist: Shreyas Shreenivas Vasanawala, MD - 10/31/2024 4:13 PM

I, the attending signed below, have personally reviewed the images and agree with the report transcribed above.

Interpreted by Attending Radiologist: Shreyas Shreenivas Vasanawala, MD

Electronically Signed: 10/31/2024 4:13 PM

Authored By : Shreyas Shreenivas Vasanawala, MD

Approval Date : 10/31/2024 4:13 PM